



14 March 2025

National Science Foundation
Networking and Information Technology Research and Development National Coordination
Office
Office of Science and Technology Policy

Submitted Electronically

The National Association for Behavioral Healthcare (NABH) is pleased to provide recommendations on the development of an artificial intelligence (AI) action plan by responding to the request for information issued that the National Science Foundation issued and [Executive Order 14179](#) required.

NABH supports the effort to develop a coordinated action plan for AI, and we strongly recommend that the plan consider the needs of behavioral healthcare service providers, so they, in turn, can protect patients with mental illness and/or substance use disorders and delivery first-rate treatment.

NABH members provide the full continuum of behavioral healthcare services to children, adolescents, adults, and older adults with mental health and substance use disorders in inpatient behavioral healthcare hospitals and units, residential treatment facilities, partial hospitalization and IOPs, medication-assisted treatment centers, specialty outpatient behavioral healthcare programs, and recovery support services in 49 states and Washington, D.C.

We recommend strongly that a national plan recognize behavioral healthcare challenges affect every American household either directly or indirectly. In addition, this letter highlights important contextual issues that national policymakers must consider.

America's Behavioral Healthcare Crisis Continues

Deaths due to suicide remain far too frequent. The American Foundation for Suicide Prevention reports that nearly 50,000 Americans died by suicide and 1.6 million Americans attempted suicide¹ in 2022. Opioid-related deaths also continue to plague the nation: the Centers for Disease Control and Prevention estimates 90,000 Americans died from an opioid overdose for the 12-month period ending August 2024. Overuse of synthetic opioids, in particular fentanyl, prescription pain relievers, and heroin continues to end lives. Unfortunately, waitlists for mental health and substance use disorder treatments remain common and, often, lengthy. In addition, the Johns Hopkins Bloomberg School of Public Health estimated economic costs associated with the opioid crisis exceeded \$1 trillion in 2024.²

¹ <https://afsp.org/suicide-statistics/>

² <https://www.cbsnews.com/news/drug-overdose-deaths-opioid-crisis-substance-use/>



The Behavioral Healthcare Field Uses Outdated Information Technology (IT)

The behavioral healthcare field's IT capacity continues to lag a generation behind those of our physical healthcare peers. This gap is largely due to behavioral healthcare providers' exclusion from *The Health Information Technology for Economic and Clinical Health Act* (HITECH). As the HHS Assistant Secretary for Technology Policy/ Office of the National Coordinator for Health Information Technology and the Substance Abuse and Mental Health Services Administration recognized through their joint Behavioral HIT Initiative in January 2024, our field today has much lower EHR adoption rates, as well as a lack of EHR developers creating IT systems for the behavioral healthcare patient population.

Today, most inpatient psychiatric facilities (IPFs) and other behavioral healthcare providers lack the capacity to exchange patient health information interoperably with external clinical partners. **Also, while some psychiatric hospitals and units have HIT systems that comply with the 2015 HHS standards for data exchange and other required functionality, most own non-compliant systems that are used primarily to bill payers electronically, with some capability for basic prescription management. However, many lack the capability to send or receive interoperable data. In fact, many in our field still rely, at least in part, on faxes, emails, and phone calls.**

AI in Behavioral Healthcare Is Growing and Will Continue to Become More Important

This request for information specifically calls for policy recommendations related to multiple AI priority areas³. As confirmed by two 2024 AI surveys of our members, we understand traditional behavioral healthcare providers are not leading AI development. Therefore, rather than addressing these technical issues, we submit comments to raise awareness and cautions for AI policymakers about what smaller healthcare providers – including rural healthcare providers and tribal organizations – require to ensure AI is implemented effectively at their organizations.

To share what we know so far, we see that our members are focused presently on using AI to improve care quality by:

- augmenting the role of diagnostic and other decision-making by clinical practitioners;
- raising the efficiency of business operations such as medical record documentation-related functions; and
- improving workforce retention through related reduction in administrative burden.

With regard to the scope of AI rollout, our AI survey results indicate that one-third or fewer of our members are using AI. **In addition, we note the growing use of telephone apps and online services being used, in part with AI functions, to provide behavioral healthcare services**

³ RFI's Specified AI Topics: Hardware and chips, data centers, energy consumption and efficiency, model development, open source development, application and use, explainability and assurance of AI model outputs, cybersecurity, data privacy and security throughout the lifecycle of AI system development and deployment relative to cyberattacks, risks, regulation and governance, technical and safety standards, national security and defense, research and development, education and workforce, innovation and competition, intellectual property, procurement, international collaboration, and export controls.



that appear, at least in part, to fall outside of evidence-based care. These platforms can be purchased by individuals who want support related to actual and perceived depression, anxiety, PTSD, psychosis and ADHD. We note that some of these prescription digital therapeutics are subject to FDA review.

Cybersecurity Concerns

On March 7, NABH submitted comments to the U.S. Health and Human Services Department's Office of Civil Rights about a [proposed rule](#) to modernize the provisions in the *Health Insurance Portability and Accountability Act of 1996* (HIPAA) to strengthen cybersecurity protections for electronic protected health information (ePHI). In general, our [comments](#) on this rule expressed our appreciation for the efforts to strengthen protections against ever-increasing cybersecurity threats. **In addition, our comments cautioned that the proposed rule did not account for the limited HIT capacity in the behavioral healthcare field.** For example, the proposal's suggestions about risk assessment, planning for future security incidents, and multiple additional technical provisions, all exceed the capacity of the majority of behavioral healthcare providers.

We note our cybersecurity letter to highlight our strong recommendation, which also is equally pertinent to this RFI, that policymakers consider equally important and less-resourced stakeholders when building modern infrastructure for the country. To this end, NABH is eager to facilitate communication between behavioral healthcare leaders and your office's effort to develop a national AI plan.

Thank you for considering NABH's recommendations about this pending action plan. We look forward to supporting and working with you and your staff to address these issues. Please contact me if you have questions.

Sincerely,

Shawn Coughlin
President and CEO